

SRA-001 Module 1: Introduction to Hijama

Student Workbook

Student Name: _____

Cohort: _____

Date: _____

Module Overview

This workbook accompanies Module 1 of the SRA-001 Certified Hijama Practitioner programme. Complete all exercises, case studies, and reflection prompts to reinforce your learning and prepare for assessment.

Learning Objectives Checklist

By the end of this module, you will be able to:

- Define Hijama (wet cupping) with clinical precision
 - Distinguish between wet cupping, dry cupping, and other modalities
 - Explain the regulatory status of cupping therapy in key jurisdictions
 - Articulate the professional identity and scope of practice of a CHP
 - Apply the concepts of amanah and ihsan to clinical practice
 - Describe the SRA Competency Framework and its six domains
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Exercise 1: Definitions and Distinctions

1.1 Write the clinical definition of Hijama (wet cupping) in your own words:

1.2 Complete the comparison table:

Feature	Wet Cupping (Hijama)	Dry Cupping
Incision required?		
Blood extraction?		
Primary mechanism		
Evidence base		
Scope in CHP practice		

1.3 List three other cupping modalities and one key difference from Hijama:

1. _____
2. _____
3. _____

Exercise 2: The SRA Definition Framework

2.1 The four elements of the SRA Hijama Definition Framework are:

1. _____
2. _____
3. _____
4. _____

2.2 Why is each element essential? Explain what happens if one element is missing:

Exercise 3: Regulatory Awareness

3.1 Research the regulatory status of Hijama in your jurisdiction:

My jurisdiction: _____

Regulatory status: _____

Governing body: _____

Key requirements for practice:

3.2 What professional insurance do you need to practise Hijama in your jurisdiction?

Exercise 4: Scope of Practice

4.1 List five activities a CHP is PERMITTED to do:

1. _____

2. _____
3. _____
4. _____
5. _____

4.2 List five activities a CHP is PROHIBITED from doing:

1. _____
2. _____
3. _____
4. _____
5. _____

4.3 Scenario: A patient asks you to diagnose their chronic fatigue. What do you do?

Exercise 5: The Amanah Framework

5.1 Define amanah in the context of Hijama practice:

5.2 The five pillars of the SRA Amanah Framework:

Pillar	What it means in my practice
Clinical Competence	
Intellectual Honesty	
Patient Safety Priority	
Confidentiality and Respect	
Continuous Improvement	

5.3 Write one specific commitment you make for each pillar:

1. Clinical Competence: _____
2. Intellectual Honesty: _____
3. Patient Safety Priority: _____
4. Confidentiality and Respect: _____
5. Continuous Improvement: _____

Exercise 6: Ihsan in Practice

6.1 Define ihsan:

6.2 How does the Hadith of Gabriel relate to clinical practice?

6.3 Describe three ways you will practise ihsan in your Hijama practice:

1.

2.

3.

Exercise 7: The Competency Framework

7.1 The six domains of the SRA Competency Framework:

1.

2.

3.

4.

5.

6.

7.2 What level of competency is required for CHP certification?

7.3 Self-assessment: Rate your current level (1-4) in each domain:

Domain	My Current Level (1-4)	Evidence
Knowledge (Ilm)		
Clinical Assessment		
Technical Skills (Itqan)		
Safety Management (Hifz)		
Communication (Kalam)		
Professionalism (Amanah)		

Case Study 1: The Uncertain Patient

Scenario:

Amina, 34, comes to your clinic asking for “cupping” because her friend recommended it for back pain. She says she wants “the one where you cut the skin and take the bad blood out.” She has never had Hijama before and is nervous about the procedure.

Questions:

1. **Patient Communication:** How do you explain what Hijama is in accessible language?

2. **Education:** What distinctions do you need to clarify about wet vs. dry cupping?

3. **Consent:** What information must Amina receive before she can give informed consent?

4. **Scope:** Amina asks if Hijama will cure her back pain permanently. How do you respond?

Case Study 2: The Boundary Test

Scenario:

Khalid, 45, has been your patient for three months. He has Hijama monthly for general wellness. After his last session, he messages you asking if you can prescribe him herbal supplements for his high blood pressure. He says he doesn’t trust conventional medicine.

Questions:

1. **Scope Boundary:** Is prescribing herbal supplements within your scope as a CHP?

2. **Ethical Response:** How do you respond to Khalid’s request while maintaining the therapeutic relationship?

3. **Referral:** What is your responsibility regarding his high blood pressure?

4. **Amanah:** How does the Amanah Framework guide your response?

Reflect–Learn–Apply Journal

Reflection Prompt 1: Amanah

“Indeed, We offered the Trust to the heavens and the earth...” — Qur’an 33:72

Reflect: What does amanah (sacred trust) mean to you personally in the context of becoming a Hijama practitioner?

Learn: What is one new insight you gained about the responsibility of a practitioner from this module?

Apply: What is one specific action you will take in your first month of practice to honour the amanah?

Reflection Prompt 2: Ihsan

“Worship Allah as if you see Him...” — Sahih Muslim

Reflect: Describe a time when you witnessed ihsan (excellence) in someone else’s work. What made it excellent?

Learn: How does ihsan transform Hijama from a technical procedure into a spiritual practice?

Apply: What is one area of your practice where you will commit to ihsan from day one?

Knowledge Check: Module 1

Multiple Choice

1. Which of the following is the correct clinical definition of Hijama?

- a) Any form of cupping therapy
- b) Controlled incision + negative pressure + blood extraction
- c) Suction-only therapy without skin puncture
- d) Fire-based heating of cups on the skin

Answer: _____

2. A CHP is permitted to:

- a) Diagnose medical conditions
- b) Prescribe medication
- c) Perform wet cupping within scope
- d) Advise patients to stop conventional treatment

Answer: _____

3. The SRA Competency Framework requires what level for CHP certification?

- a) Level 1: Foundational
- b) Level 2: Developing
- c) Level 3: Competent
- d) Level 4: Proficient

Answer: _____

4. Which of the following is NOT one of the five pillars of the Amanah Framework?

- a) Clinical Competence
- b) Financial Gain
- c) Patient Safety Priority
- d) Continuous Improvement

Answer: _____

Short Answer

5. Explain the difference between wet cupping and dry cupping in two sentences:

6. Why is the concept of amanah central to Hijama practice?

7. What does ihsan mean, and how does it apply to clinical practice?

Module 1 Summary: Key Facts

#	Key Fact
1	Hijama = wet cupping: incision + suction + blood extraction
2	Dry cupping = suction only, no incision —a distinct modality
3	CHPs work within defined scope; cannot diagnose or prescribe
4	Regulatory status varies by jurisdiction; insurance is mandatory
5	Amanah = sacred trust; the patient entrusts their body to your care
6	Ihsan = excellence in awareness of Allah; transforms practice into worship
7	Six competency domains: Knowledge, Assessment, Technical Skills, Safety, Communication, Professionalism
8	Level 3 (Competent) required in all domains for CHP certification

Further Reading

Islamic Sources:

- Qur'an 33:72 — The Trust (al-Amanah)
- Qur'an 4:58 — Rendering trusts to whom they are due
- Sahih Muslim, Hadith 8 — The Hadith of Gabriel (on Ihsan)
- Sahih al-Bukhari, Hadith 5371 — “The best of remedies is Hijama”

Clinical Sources:

- SRA Clinical Practice Standard (SRA-CPS-001)

- SRA Code of Ethics (SRA-CE-001)
- SRA Competency Framework (SRA-CF-001)

Recommended:

- Ibn al-Qayyim al-Jawziyya, *Zad al-Ma'ad* — Prophetic guidance on Hijama
- Cramer et al. (2022), *Systematic Review of Cupping Therapy* (PMID: 35180526)

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